

Pediatric P-03 Pediatric Respiratory Distress

Priorities

- Place child in position of comfort. Enlist help of parent/guardian if distress is mild or moderate
- Early notification of receiving hospital

Asthma/Wheezing

Secure airway using BLS airway adjuncts if necessary

SpO₂

Oxygen – titrate to SpO₂ >95%

Cardiac Monitor

Albuterol 5mg via HHN/nebulizer mask/CPAP
May repeat x1 for continued wheezing

Consider IV/IO Access

For Severe Distress

Epinephrine 1:1,000 0.01mg/kg IM
Max single dose 0.3mg IM

Consider CPAP for patients ≥12 years old

Stridor

Secure airway using BLS airway adjuncts if necessary. Do not visualize or insert anything into mouth/throat if epiglottitis is suspected.

SpO₂

Oxygen – titrate to SpO₂ >95%

Cardiac Monitor

Minimize outside stimulation.
Keep patient in position of comfort or sit upright.

Consider **nebulized saline**

Nebulized epinephrine
1:1,000 0.5mL/kg to max 5mL
via HHN/nebulizer mask/BVM

For doses <5mL, mix with NS
to create 5mL volume

General SOB

Secure airway using BLS airway adjuncts if necessary

SpO₂

Oxygen – titrate to SpO₂ >95%

Cardiac Monitor

Consider IV/IO access

Foreign Body Airway Obstruction (FBAO)

Mild FBAO

Assess and support ABCs
Oxygen and suction as necessary

Severe FBAO

Patients <1 year old – 5 back blows and 5 chest thrusts

Patients ≥1 year old – abdominal thrusts

Unconscious patients – CPR

Finger sweep only if object is visualized

Direct laryngoscopy and
remove obstruction with
Magill forceps

If unable to clear airway and adequately ventilate

Consider needle
cricothyrotomy

DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.